



ISPD Asia-Pacific Chapter Newsletter, April 2026

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Prepared by Chia-Te Liao and Talerngsak Kanjanabuch

News from the ISPD

Message from ISPD-APC Chapter Convenor

As we move into April, the ISPD Asia-Pacific Chapter continues to gain strong momentum- translating vision into concrete, high impact progress across the region.

A major milestone comes from Taiwan, where the **Home Dialysis White Paper (2026-2035)** sets a bold target of 18% home dialysis prevalence by 2035, signaling a decisive shift toward home-centric, sustainable kidney care. In parallel, South Korea's expanding **PD Home Management Program**, now reaching over 8,000 patients nationwide, exemplifies how structured systems, standardized training, and policy alignment can drive a meaningful resurgence of PD.

Across South Asia, momentum is equally encouraging. **Bangladesh's 2nd PD CON 2025** highlights growing engagement in education and advocacy, while activities at the **Pakistan Society of Nephrology Conference 2026** (30 January - 1 February) demonstrated strong demand for PD capacity building, with oversubscribed workshops and high-impact academic exchange.

Our investment in people continues to deliver results, The first ISPD-APC Fellowship 2025 cohort - including **Dr. Rubash Nath Yogi** (Nepal) and **Dr. Qiqi Yan** (China) - have shown the power of structured training. Notably, Dr. Rubash published two articles during the fellowship and received a WCN 2026 Travel Grant, underscoring the program's academic impact.

We are also pleased to introduce enhancements to the ISPD websites (<http://ispd.org/>), offering improved access to education, resources, and global collaboration.

Together, these developments reflect a region that is not only advancing - leading. Thank you for your continued dedication, and I look forward to building the next phase of progress with you.

Warmest regards,

Professor Talerngsak Kanjanabuch, M.D.

ISPD-APC Chapter Convenor and ISPD Council Member



Professor
Talerngsak
Kanjanabuch, M.D.

ISPD Asia-Pacific Chapter Scholarship Application

This is a scholarship to support **up to 3 months** training in clinical PD for doctors and nurses from Asia-Pacific region. The purpose is to promote PD awareness, knowledge, and expertise by visiting a center of excellence.

Applicants must be **ISPD members**.

- For medical applicants, candidates who have completed their nephrology training and with some working experience as a junior consultant would be preferred.
- For applicants of other healthcare professionals, candidates who have a working experience of at least 5 years would be preferred.

Each application must include the following documents: (1) A completed application form along with (2) a curriculum vitae, (3) a recommendation letter from the director of the nephrology unit or department, (4) a letter

of acceptance from the institution to be visited; and (5) a motivation letter.

A letter of support from the administrator of the applicant's institution is advisable.

The next deadline is **30 June 2026**.

Details and application procedures can be found under the Regional Chapters – Asia-Pacific Chapter, at the ISPD website. [Asia-Pacific Chapter - International Society for Peritoneal Dialysis](#)

Renew your membership!

Individual memberships

- For Trainees and Fellows: A very discounted membership for USD \$60 per year (after providing proof of status).
- For Doctors: Full membership for one year costs USD \$175, but there are great discounts for 2- and 3-year membership packages. Moreover, in 2026 we introduced a new discounted membership fee for all Low Income Countries (LIC) and Lower-Middle Income Countries (LMIC) at USD \$60 per year!
- For Nurses/AHPs: Full membership for one year costs USD \$100, and there are also discounted packages for 2- and 3-year membership packages, and nurses/AHPs in Low Income Countries (LIC) and Lower-Middle Income Countries (LMIC) can also enjoy a reduced membership fee of only USD \$60 per year.
 - An Associate Nurse/AHP option is also available for \$15 USD per year (with limited benefits).
- Retired Professionals can enjoy a discounted membership for \$80 USD per year (after providing proof of status).

Group memberships

- Society Membership: Your national society can contact the ISPD and negotiate a discounted price for groups of members.
- Institutional/Collective memberships: in low and middle income countries, groups of up to 10 members can join in a combined membership for only USD \$175.

Visit <https://ispd.org/memberships/> to join the ISPD or renew your membership.

Membership benefits of the ISPD include:

- Subscription to Peritoneal Dialysis International
 - ISPD Newsletter sent every two months and ISPD APC Newsletter sent every 4 months
 - Direct access to all ISPD Educational materials: ISPD Guidelines, Webinars, Journal Clubs, Case Discussions...
 - Special registration fees at ISPD events, including the ISPD APC Meeting in September 2027 in Busan (Korea) and the ISPD 2026 Congress in Cape Town.
 - Opportunity to apply for ISPD Scholarships and Grants
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Upcoming Meetings

ISPD 2026 Congress

29 September - 3 October 2026

Cape Town International Convention Centre (CTICC), Cape Town, South Africa

Early Registrations: Until **30 April 2026**

You can review the program [HERE](#) and meet the speakers [HERE](#).

More information: [ISPD 2026 website](#)



ISPD APCM 2027

1 October – 3 October 2027

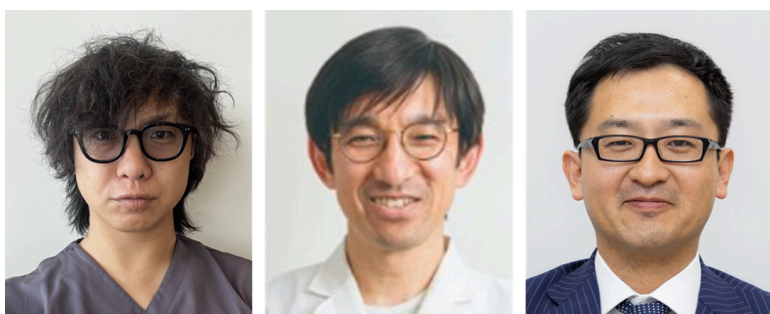
Busan, South Korea

More information coming soon



Research News from Asia-Pacific Region

First step in exploring the role of SGLT2 inhibitors for patients on dialysis



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Ultrafiltration, Volume status in PD

Peritoneal dialysis (PD) is an established renal replacement therapy, based on the exchange of water and solute between capillary blood and dialysate fluid across the peritoneal membrane. Volume overload is common in patients on PD and contributes to cardiovascular complications, technique failure, and mortality. Ultrafiltration in PD depends on the osmotic gradient typically induced by the hypertonicity of the dialysate, which is generated by osmotic agents. The glucose gradient peaks at the start of dialysis but diminishes as glucose diffuses into blood. The osmotic gradient induced by icodextrin diminishes slowly and is maintained longer than that of glucose solution, resulting in greater ultrafiltration. However, its use is limited because side effects such as aseptic

peritonitis and skin rash have been reported in the early stages when it became available for clinical use. One typical strategy to achieve a steeper osmotic gradient is to use a PD solution with higher glucose concentrations. However, this strategy might be ineffective because of increased fluid intake attributable to hyperglycemia, and it can lead to more rapid damage to the peritoneal membrane, resulting in the discontinuation of PD therapy.

Ultrafiltration and SGLT2

The mechanisms of glucose absorption across the peritoneal membrane are not fully understood. Although diffusion has been considered the primary pathway, recent studies have demonstrated the expression of sodium–glucose cotransporter 2 (SGLT2) in the human peritoneum [1]. SGLT2 inhibitors block glucose reabsorption by inhibiting this transporter, which is primarily expressed in the renal proximal tubules. Animal studies demonstrated that SGLT2 inhibitors increased ultrafiltration, potentially by maintaining the glucose gradient through the inhibition of peritoneal SGLT2 activity [2].

EMPOWERED Trial

We investigated a novel strategy for fluid management in peritoneal dialysis using an SGLT2 inhibitor. Based on the hypothesis that peritoneal SGLT2 contributes to glucose absorption from dialysate, its inhibition could preserve the glucose gradient and enhance ultrafiltration. Although observational studies have suggested increased ultrafiltration volume (UFV) with SGLT2 inhibitors, findings have been inconsistent.

The EMPOWERED trial was a multicenter, randomized, double-blind, placebo-controlled, crossover trial conducted at four Japanese hospitals to evaluate the effect of SGLT2 inhibitors on ultrafiltration in patients undergoing PD [3]. This study represents the first randomized controlled trial to evaluate the efficacy of SGLT2 inhibitors in the field of dialysis. Patients with chronic heart failure using ≥ 3 L/day of glucose-based PD solutions were eligible, regardless of diabetes status. Participants were randomly assigned (1:1) to receive empagliflozin 10 mg once daily and then placebo or vice versa. The primary endpoint was the change in the daily ultrafiltration volume from baseline to week 8 in each intervention period.

Thirty-seven participants were analyzed. After 8 weeks, the mean change in daily UFV was -43 mL (95% CI: -101 to 16) with empagliflozin and -5 mL (95% CI: -60 to 50) with placebo, showing no significant between-treatment difference (-38 mL; 95% CI: -120 to 44 ; $P = 0.36$). Similarly, dialysate glucose levels remained unchanged (-20 mg/dL; 95% CI: -63 to 23 ; $P = 0.34$). Adverse events were more frequent during empagliflozin treatment compared with placebo (47% vs. 29%).

Discussion

In this trial, SGLT2 inhibitors did not significantly modify ultrafiltration or dialysate glucose concentrations, indicating that they are unlikely to enhance fluid removal in PD. Adverse events occurred more frequently during empagliflozin treatment, although causality remains uncertain. Importantly, these findings do not preclude potential clinical benefits of SGLT2 inhibitors in PD patients. As secondary outcomes, empagliflozin was associated with a numerical reduction in NT-proBNP levels (21%) and a downward trend in body weight, findings that align with its established role in heart failure prevention. Given the short intervention period and limited sample size, larger and longer-term studies are required to validate these observations.

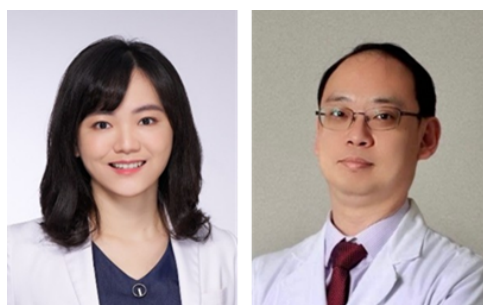
Extensive evidence from large-scale clinical trials has demonstrated that SGLT2 inhibitors reduce cardiovascular mortality and heart failure hospitalization, independent of diabetes status. These protective effects have been observed regardless of diabetes status in both heart failure with reduced ejection fraction (HFrEF) [4,5] and heart failure with preserved ejection fraction (HFpEF) [6,7]. subtypes and persist even in patients with advanced CKD and an eGFR as low as 20 mL/min/ 1.73 m². Currently, SGLT2 inhibitors are reported to have anti-fibrotic and anti-inflammatory properties [8], suggesting cardio-renal protective effects beyond their action on the proximal tubules. These findings highlight the need to evaluate the efficacy and safety of SGLT2 inhibitors in patients with end-stage kidney disease, and several trials are currently underway [9]. Our work represents an important first step in exploring the role of SGLT2 inhibitors for patients on dialysis.

References

1. Schricker S, Oberacker T, Fritz P, et al. Peritoneal expression of SGLT-2, GLUT1, and GLUT3 in peritoneal dialysis patients. *Kidney Blood Press Res* 2022;47:125-134.
2. Zhou Y, Fan J, Zheng C, et al. SGLT-2 inhibitors reduce glucose absorption from peritoneal dialysis solution by suppressing the activity of SGLT-2. *Biomed Pharmacother* 2019;109:1327-1338.
3. Doi Y, Shinzawa M, Arisato T, et al. Empagliflozin and ultrafiltration volume in patients undergoing peritoneal dialysis. *Kidney Int Rep* 2025;11:141-151.

4. McMurray JJV, Solomon SD, Inzucchi SE, et al. Dapagliflozin in patients with heart failure and reduced ejection fraction. *N Engl J Med* 2019;381:1995-2008.
5. Packer M, Anker SD, Butler J, et al. Cardiovascular and renal outcomes with empagliflozin in heart failure. *N Engl J Med* 2020;383:1413-1424.
6. Anker SD, Butler J, Filippatos G, et al. Empagliflozin in heart failure with a preserved ejection fraction. *N Engl J Med* 2021;385:1451-1461.
7. Solomon SD, McMurray JJV, Claggett B, et al. Dapagliflozin in heart failure with mildly reduced or preserved ejection fraction. *N Engl J Med* 2022;387:1089-1098.
8. Nikolic M, Zivkovic V, Joksimovic Jovic J, et al. SGLT2 inhibitors: a focus on cardiac benefits and potential mechanisms. *Heart Fail Rev* 2022;27:935-949.
9. Stepanova N. SGLT2 inhibitors in peritoneal dialysis: a promising frontier toward improved patient outcomes. *Ren Replace Ther* 2024;10:5.

Icodextrin Use and Clinical Outcomes in Peritoneal Dialysis Patients with Congestive Heart Failure: A Time-Varying Exposure Design and Target Trial Emulation Approach



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Introduction

Congestive heart failure (CHF) is a frequent and clinically important comorbidity among patients receiving peritoneal dialysis (PD), affecting up to 40% of the PD population. The coexistence of CHF and end-stage kidney disease is associated with poor survival, recurrent hospitalizations, and major challenges in long-term volume control [1]. Adequate and sustained volume control is central to improving outcomes in this high-risk population. Conventional glucose-based PD solutions rely on crystalloid osmotic gradients to achieve ultrafiltration. However, their efficacy declines during long dwells due to glucose absorption, often resulting in fluid reabsorption and adverse metabolic effects [2]. Icodextrin, a glucose polymer-based solution, provides sustained ultrafiltration through colloid osmosis and has been widely adopted for long dwells [3]. While previous studies have demonstrated improvements in fluid status, metabolic parameters, and new-onset CHF with icodextrin [4, 5], evidence linking its use to cardiovascular outcomes in PD patients with established CHF remains limited. This study aimed to evaluate the association between icodextrin use and major clinical outcomes in PD patients with pre-existing CHF [6].

Methods

We conducted a retrospective cohort study using the Chang Gung Research Database, including 1,800 adult PD patients with documented CHF who initiated PD between 2005 and 2022 and were followed through June 2023. Icodextrin exposure was defined as use for $\geq 50\%$ of total PD duration [4, 7] and was modeled as a time-varying treatment to account for treatment initiation, discontinuation, and intermittent use. Primary outcomes included all-cause mortality, cardiovascular mortality, sudden death, and major adverse cardiovascular events (MACE), and cancer incidence. Secondary outcomes included renal complications: development of encapsulating peritoneal sclerosis, transition to hemodialysis and receipt of renal transplantation. Multivariable Cox proportional hazards models incorporating time-varying covariates were applied. To evaluate the robustness of our primary analysis

and minimize immortal time and selection biases, sensitivity analyses were performed using a target trial emulation framework [8] and an alternative time-dependent data structure [9].

Results

After multivariable adjustment, icodextrin users had significantly lower incidences of all-cause mortality (adjusted hazard ratio [aHR] 0.16), cardiovascular mortality (aHR 0.20), sudden death (aHR 0.15), and MACE (aHR 0.68) compared with non-users (Table 1). The reduction in MACE was primarily driven by lower cardiovascular and sudden cardiac death rather than myocardial infarction or ischemic stroke. Icodextrin use was also associated with reduced risks of encapsulating peritoneal sclerosis and transition to hemodialysis, suggesting potential benefits in technique survival.

Table 1. Outcomes compared between icodextrin users and non-users in the primary analysis.

Outcome	Incidence ^a (95% CI)		Crude analysis		Multivariable analysis ^b	
	Exposed	Unexposed	HR (95% CI)	P-value	HR (95% CI)	P-value
All-cause death	3.2 (2.6–3.8)	14.0 (12.6–15.4)	0.22 (0.18–0.27)	<.001	0.16 (0.13–0.20)	<.001
Cardiovascular death	1.0 (0.7–1.4)	3.4 (2.7–4.1)	0.28 (0.19–0.42)	<.001	0.20 (0.13–0.30)	<.001
Sudden death	2.3 (1.8–2.8)	11.0 (9.7–12.3)	0.20 (0.16–0.26)	<.001	0.15 (0.11–0.19)	<.001
Heart failure hospitalization	5.0 (4.2–5.7)	3.7 (3.0–4.5)	1.39 (1.07–1.80)	.015	1.06 (0.80–1.40)	.702
Myocardial infarction	2.2 (1.7–2.7)	1.6 (1.1–2.1)	1.36 (0.93–2.01)	.116	0.94 (0.63–1.43)	.785
Ischemic stroke	1.8 (1.4–2.2)	1.5 (1.0–2.0)	1.21 (0.81–1.82)	.356	1.39 (0.90–2.15)	.136
MACE ^c	9.9 (8.8–11.0)	11.6 (10.3–12.9)	0.83 (0.71–0.97)	.022	0.68 (0.58–0.80)	<.001
New diagnosis of cancer	2.6 (2.1–3.1)	2.8 (2.1–3.4)	0.87 (0.63–1.19)	.385	0.88 (0.63–1.23)	.458
Encapsulated peritoneal sclerosis	5.0 (4.3–5.7)	11.1 (10.1–12.2)	0.41 (0.35–0.49)	<.001	0.40 (0.33–0.48)	<.001
Shift to hemodialysis	13.0 (11.9–14.2)	22.1 (20.7–23.5)	0.59 (0.53–0.66)	<.001	0.52 (0.46–0.58)	<.001
Renal transplantation	1.5 (1.1–1.9)	5.1 (4.3–5.9)	0.25 (0.19–0.35)	<.001	0.22 (0.16–0.30)	<.001

^aNumber of events per 100 person-year.
^bAdjusted for sex, age, body mass index, diabetes mellitus, hypertension, cardiovascular disease, LVEF, dialysis modality, admissions in the previous year, albumin, hemoglobin, high potency statin, loop diuretics, insulin, cigarette smoking, alcohol drinking and year of dialysis initiation.
^cAny of cardiovascular death, heart failure hospitalization, myocardial infarction and ischemic stroke.

Subgroup analyses demonstrated more pronounced associations among patients aged ≤65 years, those with diabetes mellitus, individuals with moderate glycemic control (HbA1c 7–9%), and patients without pre-existing cardiovascular disease. Results were consistent across sensitivity analyses, including target trial emulation, supporting the robustness of the findings.

Discussion and Clinical Implications

This study provides important real-world evidence supporting the use of icodextrin in PD patients with pre-existing CHF. By demonstrating associations with improved survival and reduced cardiovascular events, the findings extend previous evidence that focused primarily on fluid status and metabolic outcomes.

The use of time-varying exposure models and target trial emulation strengthens confidence in the observed associations and highlights the value of advanced analytical methods when randomized trials are not feasible. For clinicians caring for PD patients with CHF, these results suggest that sustained use of icodextrin for long dwells may contribute to improved long-term outcomes.

Conclusions

In this multi-institutional cohort from Taiwan, icodextrin use was consistently associated with more favorable cardiovascular and renal outcomes in PD patients with pre-existing CHF, particularly among younger individuals and those with diabetes mellitus. These findings support the consideration of icodextrin as a preferred dialysate in this high-risk population. Prospective studies are warranted to validate its long-term clinical advantages and to further confirm causality.

References

1. Ginieri-Coccossis M, Theofilou P, Synodinou C, et al. Quality of life, mental health and health beliefs in haemodialysis and peritoneal dialysis patients: investigating differences in early and later years of current treatment. *BMC Nephrol* 2008;9:14.
2. Mehrotra R, Devuyst O, Davies SJ, et al. The current state of peritoneal dialysis. *J Am Soc Nephrol* 2016;27:3238–52.
3. Mohnen SM, van Oosten MJM, Los J, et al. Healthcare costs of patients on different renal replacement modalities—analysis of Dutch health insurance claims data. *PLoS One* 2019;14:e0220800.
4. Wang IK, Chan CI, Lin AH, et al. The impact of icodextrin on the outcomes of incident peritoneal dialysis patients. *PLoS One* 2024;19:e0297688.

5. Wang IK, Lin CL, Yen TH, et al. Icodextrin reduces the risk of congestive heart failure in peritoneal dialysis patients. *Pharmacoepidemiol Drug Saf* 2018;27:447–52.
6. Ma LY, Tu YR, Chan MJ, et al. The effect of icodextrin on peritoneal dialysis patients with congestive heart failure: a time-varying exposure design and target trial emulation approach. *Nephrol Dial Transplant* 2025 Dec 13. Online ahead of print.
7. Han SH, Ahn SV, Yun JY, et al. Effects of icodextrin on patient survival and technique success in patients undergoing peritoneal dialysis. *Nephrol Dial Transplant* 2012;27:2044–50.
8. Hernan MA, Robins JM. Using big data to emulate a target trial when a randomized trial is not available. *Am J Epidemiol* 2016;183:758–64.
9. Murray EJ, Caniglia EC, Petito LC. Causal survival analysis: a guide to estimating intention-to-treat and per-protocol effects from randomized clinical trials with non-adherence. *Res Methods Med Health Sci* 2021;2:39–49.

Universal exit-site antibiotic prophylaxis reduces exit-site infections in paediatric peritoneal dialysis [1]



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Introduction

Peritoneal dialysis (PD) remains a cornerstone of kidney replacement therapy for infants, children and adolescents [2,3]. Exit-site and tunnel infections (ESI/TI), alongside peritonitis, are among the most serious complications, resulting in hospitalisation, morbidity, modality failure and even mortality [2]. Paediatric guidance from the International Society of Peritoneal Dialysis (ISPD) endorses topical antibiotic prophylaxis at the exit site [4], but the recommendation has historically rested on limited paediatric evidence [5], and is extrapolated from adult data [6-8]. Concerns over antimicrobial resistance and a potential ecological shift towards *Pseudomonas aeruginosa* have further contributed to heterogeneity in global practice [9].

Against this backdrop, a prospective cohort study by Hong Kong Children's Hospital and the Chinese University of Hong Kong evaluated the clinical impact of instituting daily exit-site antibiotic prophylaxis universally among all children and young people (<21 years) receiving chronic PD in Hong Kong. The policy, introduced on 1st January 2022, standardised the application of 2% mupirocin ointment during daily exit-site care, with gentamicin reserved for those with prior gram-negative infections and documented susceptibility. Importantly, policies pertaining to catheter placement, peri-operative antibiotic prophylaxis, exit-site care procedures (including chlorhexidine cleansing), and follow-up care bundles remained unchanged across study period, enabling a pragmatic assessment of the value of prophylaxis within a mature programme that adheres to contemporary paediatric PD standards [10].

The study spanned two periods: a two-year pre-prophylaxis period (2020–2021) and a three-year post-prophylaxis period (2022–2024). Fifty-seven children (median age at enrolment, 15.1 years, IQR 9.2–17.2; predominantly on automated PD) contributed 110 patient-years of observation. Baseline characteristics were comparable between periods. Primary outcome was ESI/ TI, reported as annualised rates per patient-year.

Results

Universal exit-site antibiotic prophylaxis was associated with a marked reduction in ESI/ TI. The annualised ESI/TI rate decreased from 0.46 (95% CI 0.29–0.68) to 0.17 (0.07–0.30) episodes per patient-year (**Figure 1**), with an incidence rate ratio (IRR) of 0.34 (95% CI 0.14–0.76; $P=0.004$), consistent with a two-thirds risk reduction. The effect was primarily driven by reduction in gram-positive infections: from 0.35 (0.20–0.54) to 0.10 (0.04–0.23)

episodes per patient-year ($P=0.007$). There were fewer infections caused by *Staphylococcus* spp.. Notably, *S. aureus* episodes halved, and no *S. epidermidis* or *S. capitis* ESI/TI occurred during the post-prophylaxis period. In contrast, gram-negative, fungal and culture-negative ESI/TI rates remained comparable across the two periods, with only a single episode of ESI/TI caused by *P. aeruginosa* was recorded after policy implementation.

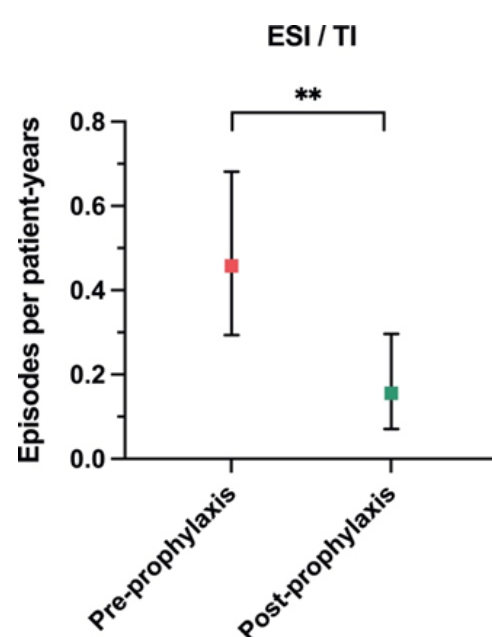


Figure 1. The annualized rates of ESI/TI during the pre-prophylaxis and post-prophylaxis periods. ESI/-TI, exit-site infection or tunnel infection. ** $P < 0.01$

Peritonitis remained infrequent (one episode pre- *versus* three post-implementation respectively), and the difference in annualised rates was also not significant. Importantly for families and healthcare utilisation, length of stay due to infective episodes were effectively halved—from 4.22 to 2.08 days per patient-year (IRR 0.49, 95% CI 0.39–0.62; $P<0.001$). With ointment costs averaging ~USD 48 per patient-year and nominal hospital bed-day costs of USD 650, universal antibiotic policy led to an estimated cost saving of ~USD 1,343 per patient-year. Catheter revisions for infection were rare and did not differ between the two periods. Microbiologically, *S. aureus* susceptibility to methicillin, gentamicin and cotrimoxazole was preserved. There was higher resistance to erythromycin, clindamycin and methicillin among coagulase-negative *Staphylococci* were observed. Sensitivity analyses showed similar improvement in ESI/TI rates after implementing prophylaxis.

Limitations and context

Several limitations should be considered when interpreting these findings. This was a single-centre study conducted within a highly standardised service where automated PD predominates, which may limit generalisability to other Asia-Pacific paediatric settings with different training structures or resource availability. The sample size was small for evaluating infrequent outcomes such as peritonitis. In addition, mupirocin was the predominant prophylactic agent, with gentamicin used only sparingly, which precludes a direct comparison between the two agents.

Conclusions and practice implications

Our work provides real-world evidence that universal daily exit-site antibiotic prophylaxis can substantially reduce Gram-positive ESI/TI and shorten infection-related length of stay over a three-year observation period in children. This is particularly relevant in Asia–Pacific settings, where many centres favour PD as the initial modality. Implementing international guidelines requires careful consideration of the local context. For instance, although current paediatric guidance recommends 2-3 times exit-site care per week [4], this frequency may be adequate across much of Asia–Pacific where persistently high temperatures and humidity increase sweating, dressing soiling and potentially infection risk. Our findings align with and strengthen paediatric guidance supporting topical antibiotic prophylaxis as part of a wider, standardised care bundle in the Asia-Pacific region. While we did not observe an emergence of antimicrobial resistance over three years, the possible selection for methicillin-resistant coagulase-negative *Staphylococci* underscores the need for continued microbiological surveillance. Multi-centre trials should refine the optimal agent, dosing frequency and clarify its impact on peritonitis and access survival in diverse care environments.

References

1. Chan EYH, Zhou D, Siu CW, et al. Universal Exit-Site Antibiotic Prophylaxis Reduces Exit-Site Infections in Pediatric Peritoneal Dialysis. *Kidney Int Rep* 2025;26;10:3884-3892.
2. Chan EY, Yap DY, Wong WH, et al. Demographics and long-term outcomes of children with end-stage kidney disease: a 20-year territory-wide study. *Nephrology* 2022;27: 171-180.

3. Chan Y, Ma AL, Tong P, et al. Chronic peritoneal dialysis in Chinese infants and children younger than two years. *Hong Kong Med J* 2016;22:365-371.
4. Warady BA, Same R, Borzych-Duzalka D, et al. Clinical practice guideline for the prevention and management of peritoneal dialysis associated infections in children: 2024 update. *Peritoneal Dialysis International* 2024;44: 303-364.
5. Chua AN, Goldstein SL, Bell D, et al. Topical mupirocin/sodium hypochlorite reduces peritonitis and exit-site infection rates in children. *Clin J Am Soc Nephrol* 2009;4:1939-1943.
6. Chow KM, Li PK-T, Cho Y, et al. ISPD catheter-related infection recommendations: 2023 update. *Perit Dial Int* 2023;43:201-219.
7. Obata Y, Murashima M, Toda N, et al. Topical application of mupirocin to exit sites in patients on peritoneal dialysis: a systematic review and meta-analysis of randomized controlled trials. *Ren Replace Ther* 2020;6:12.
8. Bernardini J, Piraino B, Holley J, et al. A randomized trial of Staphylococcus aureus prophylaxis in peritoneal dialysis patients: mupirocin calcium ointment 2% applied to the exit site versus cyclic oral rifampin. *Am J Kidney Dis* 1996;27:695-700.
9. Borzych-Dużalka D, Same R, Neu A, et al. Best Practice of peritoneal dialysis-associated gram-negative peritonitis in children: Insights from the international pediatric peritoneal dialysis network registry. *Kidney Int Rep* 2024;9:1654-1663.
10. Neu AM, Richardson T, Lawlor J, et al. Implementation of standardized follow-up care significantly reduces peritonitis in children on chronic peritoneal dialysis. *Kidney Int* 2016;89:1346-1354.

Changing the Dialysis Story in Malaysia

The Malaysian Ministry of Health (MOH) has recently amplified its commitment to peritoneal dialysis (PD), backed by increased funding, explicit targets, and a strategic policy framework. This aligns with the “PD First” policy, which was implemented by MOH in October 2022 to make PD the primary modality for suitable patients.

In January 2026, Health Minister Datuk Seri Dr Dzulkefly Ahmad has recently reinforced its commitment to expanding PD services as part of the national strategy to manage chronic kidney disease (CKD) and end-stage kidney failure (ESKD). He had announced a budget allocation of RM100 million in the national health budget for 2026 to strengthen PD services nationwide — this is a marked increase from previous allocations of around RM40 million.

The emphasis on PD reflects several health care priorities:

- Equity in access: PD can reduce barriers for patients in rural and remote areas, where access to dialysis centres is limited.
- Patient-centred care: PD allows patients greater flexibility and can support continued productivity and quality of life, especially for working individuals.
- Cost-effectiveness: PD can be less resource-intensive over time compared with in-centre haemodialysis for some patient groups.

The Ministry has set specific targets tied to the PD strategy, with the support from Malaysian Society of Nephrology and the National Kidney Foundation including:

- 50% of eligible end-stage kidney patients receiving PD at MOH facilities within the next five years (up from current adoption rates, which are around 42% in public facilities).
- A 20% nationwide target of all dialysis patients (including private sector) on PD by 2030.

The shift towards a PD-first approach aims to enhance access, promote patient autonomy, and optimise the health system’s response to the rising prevalence of end-stage kidney disease. While challenges remain — including awareness, infrastructure, and logistics — the government’s plan represents a key public health effort to improve kidney care and patient outcomes nationwide. To address these, MOH has indicated plans for patient education, psychosocial support, improving basic infrastructure, and enhancing the role of caregivers and communities in supporting PD patients.

We would like to thank ISPD President, Professor Rajnish Mehrotra and Professor Edwina Brown for their continuous support and being present during our discussion with the Health Minister in September 2025 during the APCM-ISPD congress in Kuala Lumpur.



ISPD Asia-Pacific Chapter Advances PD in Pakistan

The Asia-Pacific Chapter of the ISPD once again strengthened PD development in Pakistan at the Pakistan Society of Nephrology (PSN) Conference 2026 (Islamabad, 30 January–1 February).

Led by ISPD President Prof. Rajnish Mehrotra, Immediate Past President Prof. Edwina Brown, Prof. Stanley Fan, and Prof. Ahad Qayyum, the faculty delivered high-impact keynote lectures and hands-on training. PD sessions were oversubscribed nearly threefold — clear evidence of growing national momentum.

The 10-station PD workshop covered catheter placement, peritonitis management, POCUS in PD, PD in the ICU, and the Saving Young Lives initiative — which drew sustained engagement. Keynote talks addressed target-driven PD, local disposables, culture-negative peritonitis, and the evolving PD landscape in Pakistan.

Building on prior collaboration in Karachi (2025), this meeting reaffirmed ISPD’s ongoing commitment to capacity building in Pakistan.

Thank you, PSN — and see you in Cape Town.



Bangladesh Accelerates Its PD Momentum: Highlights from BSPD PD CON 2025, Dhaka

The Bangladesh Society for Peritoneal Dialysis marked a major milestone with its **2nd International Conference and PD Workshop 2025** (1–2 December 2025, Dhaka Club), convening over 300 delegates and reinforcing Bangladesh’s emergence as a regional leader in peritoneal dialysis (PD).

Endorsed by the International Society for Peritoneal Dialysis, the meeting brought together a strong international and regional faculty, including Talerngsak Kanjanabuch, Nauman Tarif, Urmila Anandh, Muhammad Mohsin Riaz, and Nishantha Nanayakkara, alongside national leaders such as Harun ur Rashid, Muhammad Nazrul Islam, and M.A. Samad. Together, they fostered a vibrant platform for regional exchange, collaboration, and shared implementation strategies.

The scientific program, aligned with ISPD priorities, addressed PD expansion, infection prevention, technique survival, special populations, and patient-centered care models for LMIC settings. A standout feature was the “World Café” PD workshop, an interactive format that enabled participants to exchange frontline experiences, tackle implementation challenges, and co-develop context-specific solutions.

The program featured 12 scientific presentations, interactive discussions, and a QoL-focused poster competition, with strong engagement from young nephrologists and multidisciplinary teams—highlighting a clear emphasis on capacity building and workforce development.

Momentum has translated into action, with hands-on PD training integrated into the Bangladesh Renal Association Annual Meeting (January 2026). Concurrently, BSPD is advancing policy engagement to support PD fluid access and local manufacturing—critical for supply resilience and equitable access.

Collectively, these efforts signal a coordinated national movement. Bangladesh is rapidly positioning itself as a catalyst for PD expansion in South Asia, aligned with global priorities for equitable and sustainable kidney replacement therapy.



A Turning Point: From Training to Transformation—ISPD-APC Fellowship in Thailand



Dr. Rubash Nath Yogi

Department of Nephrology and Transplant Medicine, Shahid Dharmabhakta National Transplant Center, Bhaktapur, Nepal

The 2025 International Society for Peritoneal Dialysis–APC Fellowship was not just training—it was a turning point. Over eight immersive weeks at King Chulalongkorn Memorial Hospital, under the mentorship of Talerngsak Kanjanabuch, I witnessed how peritoneal dialysis (PD), when systematized, can scale, sustain, and transform care.

At one of Asia’s leading PD centers, I moved beyond theory into practice—managing urgent-start PD, automated PD (APD), and complex inpatient cases, while observing a system built on precision, teamwork, and continuity. Here, PD is not a modality—it is a well-orchestrated system.

Conversations with global leaders—Brett Cullis, Sunita Bavanandan, and Jadej Thammatacharee—brought this into sharper focus. Thailand’s **“PD First”** policy is not just policy—it is alignment in action, where governance, financing, and care delivery converge to expand access and improve outcomes.

At Kid Dee Kidney Care Center, I saw PD at scale—community-based, patient-centered, and relentless in follow-up, integrating training, home visits, vaccination, and telemedicine into everyday care. At Chaiphum Hospital, hands-on experience in Tenckhoff catheter insertion translated knowledge into confidence.

The most enduring lesson? “Build a track record.” During this fellowship, I contributed to publications in *Medical Mycology Case Reports* and *Kidney International Reports*, and received a Travel Grant Award for the World Congress of Nephrology 2026—early milestones that reshaped my academic path.

Returning to Nepal, I carry more than skills—I carry clarity and conviction. PD is not an alternative; it is a practical, scalable solution. This fellowship was a catalyst for change, and I am committed to translating it into action.

Acknowledgements

This report is based on experiences gained during the **ISPD Fellowship Program 2025 (First Round)**, undertaken in Thailand under the supervision of Professor Talerngsak Kanjanabuch.



Figure. ISPD-APC Fellowship Training Experience in Thailand (2025)

Representative activities during the fellowship: (A) Hands-on Tenckhoff catheter insertion under supervision at Chaiphum Hospital; (B) Dissemination of fellowship experience at the 6th Conference of the Nepal Society of Nephrology (NSN); (C) Clinical exposure at King Chulalongkorn Memorial Hospital, including personalized PD training led by Piyaporn Towannang (Chief PD Nurse); (D) Meeting with international and national leaders—Prof. Talerngsak Kanjanabuch, Dr. Brett Cullis, Dr. Sunita Bavanandan, and Dr. Jadej Thammatacharee—together with Dr. Trần Tuyết Trinh (ISPD Fellow, Vietnam); and (E) Home visit observation from Kid Dee Kidney Care Center (KDKC), demonstrating community-based PD care.

A Turning Point: From Training to Transformation—ISPD-APC Fellowship in Thailand



Dr. Tran Tuyet Trinh

Center for Nephrology, Urology and Dialysis, Bach Mai Hospital, Vietnam

Following a 6-week International Society for Peritoneal Dialysis Fellowship in Thailand (September 28–November 7, 2025), I gained more than clinical skills in peritoneal dialysis (PD)—I discovered how systems, not individuals, drive scalable impact.

Under the mentorship of Professor Talerngsak Kanjanabuch, the training was immersive, rigorous, and deeply formative. It sharpened not only procedural competence but also clinical judgment, discipline, and a systems mindset—shifting my perspective from treating patients to designing care that sustains outcomes.

At King Chulalongkorn Memorial Hospital, I trained across the full PD continuum—from outpatient care to complex inpatient management. Exposure to urgent-start PD and automated PD (APD), combined with a tightly coordinated multidisciplinary model, revealed how structure ensures consistency. Regular follow-up (every 1–3 months) supported by a 24/7 hotline, alongside integrated nutritional and psychosocial care, demonstrated PD as a system of care—not just a therapy.

At Kid Dee Kidney Care Center, I saw scale in action. Managing over 500 patients, the center showed how standardization, workflow efficiency, and strong patient engagement sustain quality at volume. At Chaiyaphum Hospital, hands-on experience in Tenckhoff catheter insertion translated knowledge into confidence, grounded in precision and safety.

The insight was unmistakable: high-quality PD is reproducible when systems are right—built on protocols, teamwork, education, and proactive follow-up. This realization reframed PD for me as a practical, scalable solution for equitable kidney care.

Returning to Vietnam, I moved quickly from insight to action—strengthening post-catheter training, establishing a dedicated PD nurse role, integrating structured nutritional care, and redesigning follow-up using risk stratification. These are small steps, but collectively they signal a shift toward a more structured, patient-centered PD system.

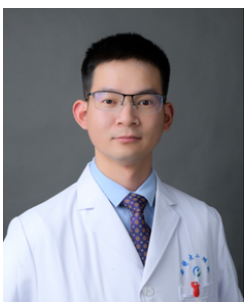
This fellowship was not just training—it was a catalyst. I return with clarity, conviction, and a commitment to scale what works. I am deeply grateful to the International Society for Peritoneal Dialysis and to Professor Talerngsak Kanjanabuch and colleagues in Thailand for this transformative experience.

Acknowledgements

This report is based on experiences gained during the **ISPD Fellowship Program 2025**, undertaken in Thailand under the supervision of Professor Talerngsak Kanjanabuch.



Reflections on the ISPD Fellowship: Advancing Peritoneal Dialysis Capacity Through Structured Training at Peking University First Hospital



Dr. Qiqi Yan

Second Affiliated Hospital of Anhui Medical University, Anhui, China

During my three months of training at the Peritoneal dialysis (PD) Center, Peking University First Hospital, I gained comprehensive insight into high-quality, goal-directed PD care. This experience not only strengthened my clinical competencies but also clarified a pragmatic and scalable pathway for advancing PD services in Anhui Province and in other regions with comparable healthcare settings.

Training Experience and System-Level Insights

Through close engagement in daily clinical practice, I observed a highly structured follow-up system and an integrated management framework. More importantly, I gained a deeper understanding of how these elements operate cohesively within a mature care model—where standardized workflows, continuous patient education, and multidisciplinary collaboration ensure both quality and efficiency. This experience reinforced a critical principle: high-quality PD care is not dependent on individual expertise alone, but on a reproducible, system-based approach. Such a model—anchored in structured follow-up, clear clinical protocols, active patient engagement, and cohesive teamwork—offers a practical and transferable blueprint for centers aiming to establish sustainable PD programs.

Academic Development and Translation Research Perspective

I was actively involved in clinical research design and implementation during my training. This experience highlighted the importance of anchoring research in real-world clinical challenges and generating evidence that is both meaningful and applicable. It highlighted the pivotal role of research as a bridge between evidence and practice. By focusing on clinically relevant questions and employing feasible yet methodologically rigorous designs, research can directly inform PD management, refine clinical decision-making, and ultimately improve patient outcomes.

Implications for Regional PD Development

This training provides several strategic directions for regional PD development. First, the establishment of standardized management systems is fundamental to ensuring quality, safety, and consistency of care. Second, strengthening technical proficiency is essential to promote confidence and uptake of PD. Techniques such as intraperitoneal pressure monitoring and individualized volume optimization offers practical solutions to reduce complications and enhance treatment outcomes. Third, the development of competency-based training programs is critical to key to building a skilled and sustainable PD workforce. Fourth, embedding patient-centered care principles can significantly improve adherence, satisfaction, and long-term clinical outcomes.

Future Directions

The ISPD-APC Fellowship represents a pivotal milestone in my professional development. Moving forward, I aim to translate these insights into practice by promoting standardized PD care, strengthening regional training capacity, and developing context-specific care pathways. I also aspire to contribute to broader academic collaboration and knowledge exchange within the global nephrology community.

Conclusion

This training has been truly transformative, enhancing both my clinical acumen and academic perspectives while fundamentally reshaping my understanding of PD care. I am deeply grateful to Professor Jie Dong and all colleagues for their invaluable mentorship and support. Although the training period was limited, its impact will be enduring, and I remain fully committed to advancing PD services both locally and regionally.

Acknowledgements

This report is based on experiences and insights gained during the **ISPD Fellowship Program 2025 (First Round)**, under the supervision of Professor Jie Dong, M.D., undertaken at Peking University First Hospital, China, from December 2025 to February 2026.



The Clinical Challenge: Urgent-Start APD with Incremental Strategy in a CKD Stage 5 Patient with Severe Hyponatremia

This case is provided by Professor KS Nayak

Background

Severe hyponatremia in CKD Stage 5 requiring urgent dialysis poses a high risk of osmotic demyelination syndrome (ODS) with conventional hemodialysis due to rapid sodium correction.

Case Presentation

A young patient with CKD Stage 5 (IgA nephropathy) presented with:

- Serum creatinine: 12.20 mg/dL
- Serum sodium: 119 mEq/L

Intervention

Urgent-start automated peritoneal dialysis with an incremental strategy (APD-IS) was initiated. A Tenckhoff catheter was placed, and APD was commenced within 4 hours of placement, bypassing the conventional 2-week break-in period due to the emergent clinical context. This approach enabled controlled solute clearance and gradual correction of serum sodium levels, minimizing the risk of rapid osmotic shifts.

Outcome

The patient demonstrated remarkable clinical improvement with zero neurological or cardiovascular complications.

- Sodium Correction: Achieved a safe, gradual increase to 128 mEq/L over 48 hours.
- Significant resolution of azotemia and clinical stability.

Conclusion

This case reinforces APD-IS as a first-line modality for patients requiring urgent dialysis where hemodynamic stability and controlled osmolar shifts are critical.

